BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Blood and organ safety	Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.	Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.	Keep a current dose and symptom record.	Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests	Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant's plan; no routine universal tumour-marker panel is specified.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00893 v1: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant’s plan; no routine universal tumour-marker panel is specified.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Methotrexate: head and neck cancer: treatment and checks

Recurrent, advanced or metastatic head and neck squamous cell cancer in people with comorbidities or unsuitable for combination chemotherapy.

Methotrexate 40 mg/m² by intravenous bolus on Days 1, 8 and 15 of each 28-day cycle, until progression or unacceptable toxicity. There is no scheduled Day-22 dose. The clinician may increase to 50 mg/m² only if there has been no mucositis.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, and chest X-ray. Review mouth symptoms, medicines, hydration and any substantial fluid collection around the lungs or abdomen.	Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Methotrexate is given into a vein by the treatment team. This is an intermittent intravenous cancer regimen, not a daily tablet schedule. Check the written appointment calendar before each visit. Never take extra methotrexate yourself.	Keep a current dose and symptom record.	Tell the pharmacist about all painkillers, antibiotics, supplements and other medicines because interactions can increase methotrexate toxicity. Report new cough or breathlessness, mouth ulcers, unusual bleeding, jaundice, reduced urine or severe rash promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	Blood tests and urinalysis assess treatment safety. Scans and cancer-specific tests follow the consultant's plan; no routine universal tumour-marker panel is specified.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1

Treatment and review

intravenous (IV) on Days 1, 8 and 15 every 28 days

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Repeat blood count, kidney and liver tests and urinalysis; examine the mouth and throat for mucosal changes during treatment. The team confirms the test timing before each planned dose and reviews recovery and previous dose delays.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.50